

1. **Occlusive barricade** : deep vein thrombosis (DVT)
2. **3-wheeled Virchow chair** : Virchow's triad (three risk factors for DVT: venous stasis, hypercoagulability, endothelial injury)
3. **Prolonged chair sitting** : venous stasis is a risk factor for DVT (e.g. hospitalization, long plane flight - or heart failure)
4. **Stick pile** : hypercoagulability is a risk factor for DVT (cancer, pregnancy, OCPs, factor V Leiden, prothrombin gene mutation, etc)
5. **Damaged endothelial tiles** : endothelial injury is a risk factor for DVT (e.g. surgery, smoking, atherosclerotic plaque)
6. **Thin vein design** : DVTs are most common in the small distal veins of the leg (causes minor lower leg pain and swelling)
7. **Proximal jacket flaps** : clinically significant DVTs form in the large more proximal veins (e.g. popliteal, femoral, iliac veins)
8. **Flying buttons** : DVTs in large proximal veins are most likely to embolize
9. **Fiery heat** : acute DVT presents with warmth
10. **Cringing** : acute DVT presents with pain
11. **Swollen pant leg** : acute DVT presents with leg swelling and edema
12. **WELL** : Wells score is used to determine if HIGH or LOW risk for DVT (based on risk factors [i.e. venous stasis, hypercoagulability, endothelial injury] and physical exam findings [swelling, edema, pain, etc])
13. **Scattered double-"D" twigs** : DVT presents with an elevated D-dimer (a measure of fibrin degradation products [particularly dimerized fibrin])(NOT specific for DVT)
14. **Bullhorn** : DVT is diagnosed with ultrasound of the leg veins
15. **Heparin hunter** : anticoagulation (heparin, warfarin, etc.) is the treatment for DVT (and PE)
16. **Wire chimney frame** : an inferior vena cava filter (IVC filter; intercepts clots in IVC) can be placed to treat DVT if anticoagulation is contraindicated (e.g. high bleeding risk: GI bleeds, intracranial tumors)
17. **Grandfather clock** : chronic DVT (recurrent DVT or residual thrombus)
18. **Bulging pipe** : post-thrombotic syndrome is a complication of chronic DVT (venous hypertension and valvular incompetence → chronic venous insufficiency)
19. **Leaky venous pipe** : chronic venous insufficiency presents with edema
20. **Peeling red wall** : chronic venous insufficiency presents with skin pigmentation and venous stasis dermatitis (skin peeling due to ulcerations)
21. **Fibrotic weeds;** chronic venous insufficiency can cause lipodermatosclerosis (fibrosis of subcutaneous tissue; starts at medial ankle)



22. **Thrombotic nest disrupting pulmonary branches** : pulmonary embolism (PE) (thromboembolism that has traveled through the venous system → through right heart → lodges in pulmonary vasculature)

23. **"Rue Iliofemoral"** : more proximal DVTs (popliteal, femoral, iliac veins) are more likely to lead to PE

24. **Big "Vive" over small "republique"** : PE causes INCREASED V-Q ratio (lung distal to the occlusion has decreased perfusion (Q), but normal ventilation (V)

25. **Distributing waste away from thrombotic nest** : in a PE, blood that would normally perfuse the obstructed region is redistributed to other lung areas

26. **Blue faced effigy** : PE causes hypoxemia (low PaO2 due to decreased perfusion to occluded lung areas)

27. **"A" roof towering over "a" effigy** : in PE, there is an increased A-a gradient (because Alveolar pO2 is high & arterial pO2 is low)

28. **Overwhelmed blowing air** : PE causes HYPERVENTILATION (manifests as tachypnea and dyspnea) (activation of chemoreceptors and pulmonary irritant sensors → activation of respiratory centers)

29. **Blowing "OH" bubbles** : PE causes RESPIRATORY ALKALOSIS (hyperventilation → "blowing-off" CO2 → decreased PaCO2 and increased arterial pH)

30. **Wedged black hat** : a PE that affects the lung periphery can cause infarction (perfusion from pulmonary arteries is crucial in periphery due to decreased prominence of bronchial arteries)(causes a wedge-shape of necrosis)

31. **Shark-tooth necklace** : pulmonary infarcts (secondary to PE) can cause pleuritic chest pain (sharp pain aggravated by inspiration)

32. **Bloody rag** : pulmonary infarcts (secondary to PE) can cause hemoptysis (due to capillary damage)

33. **"Hampton"** : pulmonary infarcts (secondary to PE) can cause a Hampton's hump on CXR (peripheral wedge shaped opacity)

34. **"Westermarque"** : pulmonary infarcts (secondary to PE) can cause a Westermark sign on CXR (hyperlucent [dark] area distal to embolus due to reduced blood flow)

35. **Tense vascular chest chains** : severe PE causes increased pulmonary vascular resistance (due to occlusion of pulmonary artery)(causes acute pulmonary hypertension)

36. **Horse saddle** : a "saddle embolus" (large PE straddling the pulmonary artery bifurcation [occludes both left and right pulmonary arteries]) can cause hemodynamic collapse

37. **CORKed heart bottle** : a large PE can cause COR PULMONALE (RIGHT heart failure secondary to pulmonary hypertension)

38. **Raised heart watch** : a large PE can cause tachycardia (RIGHT heart failure → diminished filling of LEFT ventricle → decreased cardiac output → compensatory tachycardia)

39. **Lightning bolt** : a large PE can cause cardiogenic shock (decreased cardiac output → hypotension)

40. **Black cat running down spiral staircase** : PE can be diagnosed with spiral CT angiography

41. **Heparin hunter** : anticoagulation (heparin, warfarin, etc.) is the treatment for DVT (and PE)

42. **Barricade buster** : thrombolytic agents (e.g. tissue plasminogen activator [tPA]) can be administered to dissolve the thrombus in severe PE

43. **Grandfather clock surrounded by tense arterial net** : chronic thromboembolic pulmonary hypertension can be a complication of recurrent PEs (leads to increased risk of RIGHT heart failure